

Guidelines for the diagnosis and treatment of dermatitis herpetiformis

Abstract

Dermatitis herpetiformis is a rare disease that should be considered the cutaneous expression of a gluten-sensitive enteropathy indistinguishable from celiac disease. Dermatitis herpetiformis is often misdiagnosed, and to date, no guidelines for its management have been published in the literature. These guidelines have been developed for dermatologists by the Group for Cutaneous Immunopathology of the Italian Society of Dermatology and Venereology. They reflect the best available evidence at the time of preparation, along with the clinical experience of the authors and members of the Italian Group for Cutaneous Immunopathology.

The diagnosis of dermatitis herpetiformis is established through clinical, histological, immunopathological, and serological evaluation. A gluten-free diet (GFD) is the treatment of choice. Dapsone and/or other medications may be used during the initial phase until the GFD becomes effective. These guidelines provide evidence-based recommendations for the diagnosis and management of dermatitis herpetiformis.

Keywords

celiac disease, dermatitis herpetiformis, diagnosis, guidelines, management, therapy

Introduction

Dermatitis herpetiformis is a chronic, relapsing inflammatory skin disease characterized by pruritic, polymorphic skin lesions and distinct histopathological and immunopathological features. Increasing evidence supports that dermatitis herpetiformis represents the cutaneous manifestation of a gluten-sensitive enteropathy identical to celiac disease. Both conditions are multifactorial, involving genetic predisposition and environmental triggers, resulting in specific pathology in the small intestine and skin, respectively.

There is a strong association with HLA-DQ2 (present in 80–90% of patients) and HLA-DQ8 (in 10–20% of patients), which is shared by both dermatitis herpetiformis and celiac disease.

The disease typically manifests in the third decade of life, although it can occur at any age. It predominantly affects Caucasians. The reported incidence is 11.5 per 100,000 in Scotland and ranges from 19.6 to 39.2 per 100,000 in Sweden.

Patients with dermatitis herpetiformis have an increased risk of immunomediated diseases and associated conditions, including lymphoma.

Diagnostic Protocol

Clear and concise diagnostic criteria are essential for accurate identification of dermatitis herpetiformis, ensuring appropriate treatment and long-term monitoring.

Diagnosis is based on clinical presentation, histology, immunopathology—specifically direct immunofluorescence (DIF)—and serology, including IgA anti-tissue transglutaminase (anti-tTG) and IgA endomysial antibodies (EMA).

While a suggestive clinical picture and positive serologic tests support the diagnosis—and some authors consider anti-tTG testing sufficient—DIF of perilesional (uninvolved) skin remains the gold standard for confirming dermatitis herpetiformis.

In cases of negative DIF results, serial sectioning of the biopsy specimen should be performed. If results remain negative, a second biopsy should be obtained

from clearly uninvolved skin, ensuring the patient is not following a gluten-free diet, which may affect test outcomes. Given that false-negative IgA detection is often due to technical issues, DIF should be performed in experienced laboratories to reduce the risk of both false-positive and false-negative results.